

Sepsis DRG 872 → 871: Paid-but-Downgraded Recovery

Clinical-coding crossover reasoning for a DRG downgrade dispute. Designed to complement CDI workflows, not replace them.

← ALL SPECIMENS

GET STARTED

SEND A CASE

Specimen only. Fictional case. No PHI. Demonstrates the DRG Downgrade Challenge Memo format — the deliverable a denial or CDI team receives for paid-but-downgraded claim disputes. Clinical consulting only; coding decisions remain with the client's certified coding professionals.

CASE SNAPSHOT

Claim Summary

BILLED DRG	871 — Septicemia or Severe Sepsis without MV >96 hours, with MCC
PAID DRG	872 — Septicemia or Severe Sepsis without MV >96 hours, without MCC
PLAN TYPE	Commercial PPO
PAYER ACTION	Paid at DRG 872; MCC designation removed on post-payment audit (AKI reclassified as not meeting MCC criteria)

DISPUTED
DELTA

Illustrative: \$12,000–\$18,000 depending on facility rate

PAID AT

872

Septicemia/Severe Sepsis w/o MV >96h, **without MCC**

Lower relative weight



SHOULD BE

871

Septicemia/Severe Sepsis w/o MV >96h, **with MCC**

Higher relative weight

\$12K–

\$18K

Illustrative payment delta between DRG 872 and DRG 871. These claims never appear in denial dashboards because they were paid — just at the wrong rate.

APPEALABILITY

Recovery Verdict

Strong Appealability

The removal of the MCC designation appears clinically unsupported based on the documented record. The record documents acute kidney injury (AKI) with creatinine rise from baseline 1.1 to peak 3.4 mg/dL during hospitalization, supporting the clinical significance of AKI for CDI/coding review by the client's certified coding professionals. The payer auditor's rationale — that the creatinine elevation was "transient and resolved" — applies a clinical recovery standard rather than the coding standard, which captures the condition as present and treated during the encounter regardless of resolution at discharge.

PAYER POSITION

Auditor's Downgrade Rationale

The payer's post-payment audit downgraded DRG 871 to 872 on the following basis:

- The acute kidney injury was "transient" — creatinine peaked at 3.4 mg/dL on Day 3 but returned to 1.6 mg/dL by discharge.
- No dialysis was required, and the patient was not transferred to a higher level of care for renal management.
- The auditor concluded the AKI did not represent a "major" complication because it resolved with conservative management (IV fluids and medication adjustment).

The auditor's rationale conflates clinical resolution with coding classification. These are distinct standards.

CLINICAL EVIDENCE

Severity Markers Supporting MCC

The following documented clinical findings support the coding of AKI as a major complication/comorbidity during this encounter:

SEVERITY MARKER	RECORD EVIDENCE	RELEVANCE TO MCC
Creatinine trajectory	Baseline 1.1 mg/dL (prior labs, 3 months). Admission 1.8. Day 2: 2.6. Day 3: 3.4 (peak). Day 5: 2.1. Discharge: 1.6.	Greater than 2x baseline rise meets KDIGO Stage 2 AKI criteria. The magnitude of rise, not the resolution, determines whether AKI was present during the encounter.
Clinical intervention	IV fluid resuscitation adjusted specifically for renal protection. Nephrotoxic medications held (NSAID, ACE inhibitor). Fluid balance monitored. Urine output tracked hourly on Days 2–4.	These are active clinical interventions specifically targeting the AKI — not passive observation. The resource utilization pattern supports the MCC classification.
Physician documentation	Attending progress note Day 3: "AKI — likely ATN in setting of sepsis. Holding nephrotoxins. Renal dosing antibiotics. Will trend Cr and urine output. Nephrology consult if no improvement in 24 hours."	The physician identified, assessed, monitored, and actively managed the AKI as a distinct clinical problem. This meets the standard for a condition that was present and required clinical evaluation and management during the encounter.

SEVERITY MARKER	RECORD EVIDENCE	RELEVANCE TO MCC
Medication adjustments	Vancomycin renal-dosed (trough-guided). Metformin held. Lisinopril held. Ibuprofen discontinued. Gentamicin considered and rejected due to renal status.	Multiple medication decisions were made specifically because of the AKI. This represents clinical resource utilization and decision complexity attributable to the complication.
Discharge status	Creatinine 1.6 at discharge (above baseline of 1.1). Discharge instructions include nephrology follow-up in 2 weeks and repeat renal panel. Medications remain adjusted.	The AKI had not fully resolved at discharge — creatinine remained elevated above baseline. The ongoing management plan further supports the condition's clinical significance during the encounter.

REBUTTAL

Point-by-Point Response to Auditor

PAYER AUDITOR

"The AKI was transient and resolved with conservative management."

CLINOVIAN RESPONSE

The coding standard for assigning a complication/comorbidity does not require that the condition persist through discharge or require a specific intervention threshold

such as dialysis. The standard requires that the condition was **clinically present** during the encounter and **required clinical evaluation, monitoring, or management**. AKI with creatinine rise to 3.4 (KDIGO Stage 2), active intervention (fluid management, nephrotoxin hold, renal dosing, hourly urine monitoring), and physician-documented assessment and treatment plan satisfies this standard. Resolution with conservative management does not retroactively eliminate the condition from the encounter record.

PAYER AUDITOR

"No dialysis was required."

CLINOVIAN RESPONSE

Dialysis is not a prerequisite for AKI to qualify as an MCC. The MCC classification reflects the clinical severity and resource impact of the condition during the hospitalization — not whether a specific procedural intervention was performed. Many conditions classified as MCCs (e.g., respiratory failure managed with supplemental oxygen rather than mechanical ventilation, coagulopathy managed with factor replacement rather than surgical intervention) qualify on the basis of clinical significance, monitoring, and management rather than a procedural threshold.

PAYER AUDITOR

"The complication did not rise to the level of 'major.'"

CLINOVIAN RESPONSE

The distinction between MCC and CC is a coding classification defined by CMS, not a clinical severity judgment made by the auditor. AKI is classified as an MCC in the MS-DRG system when properly documented and coded. The auditor's rationale applies a clinical-impression standard ("not major enough") rather than the coding

standard ("was the condition present, documented, and did it affect management").
The record satisfies the coding standard.

CDI / CODING HANDOFF

Notes for the CDI and Coding Team

This memo provides the clinical reasoning that supports the MCC classification. The following operational notes are for the CDI specialist and coding professional completing the appeal:

- **Verify the principal diagnosis code assignment.** Confirm the sepsis sequencing follows current guidelines — sepsis as principal diagnosis, AKI as secondary diagnosis with appropriate code reflecting the stage/severity.
- **Confirm physician query if needed.** If the attending's documentation of AKI is present but does not explicitly link it to sepsis as a complication, a compliant physician query may strengthen the record. The progress note on Day 3 already documents the clinical link ("AKI — likely ATN in setting of sepsis"), which is strong.
- **Attach the full creatinine trend.** The trajectory from baseline through peak to discharge is the single most persuasive piece of evidence. Present it as a table or timeline, not buried in lab reports.
- **Reference coding guidance.** The appeal should cite applicable coding conventions regarding the assignment of conditions that are clinically present and managed during the encounter, regardless of resolution at discharge. Clinovian provides the clinical severity argument; coding-convention citations are the CDI/coding team's domain.

This memo complements your CDI workflow — it does not replace it. The clinical severity argument (why the AKI is real, significant, and actively managed) is Clinovian's contribution. The coding-rule argument (why the MCC assignment is correct under current guidelines) is yours. The strongest appeals combine both.

RISK ASSESSMENT

Recovery Risk Factors

RISK	SEVERITY	MITIGATION
Auditor re-applies "resolved = not major" logic	Medium	Pre-empt by citing the coding standard (condition present and managed) vs. clinical outcome standard in the first paragraph of the appeal. Make the distinction unavoidable.
Physician documentation lacks explicit "AKI" diagnosis label	Low in this case	The Day 3 note explicitly names "AKI — likely ATN." If this were absent, a compliant physician query would be needed. Not required here.
Payer argues creatinine rise was pre-existing CKD progression	Medium	Baseline creatinine of 1.1 from 3 months prior establishes a clear pre-illness baseline. The acute rise to 3.4 is distinct from any chronic trajectory. Include the baseline lab in the appeal packet.
Second-level review upholds downgrade	Low	If Level 1 fails, this case is well-suited for external review — the clinical evidence is strong and the auditor's logic has a

RISK	SEVERITY	MITIGATION
		clearly identifiable error. Build the Level 1 appeal with external review in mind.

RECOMMENDATION

Final Recommendation

Pursue recovery. The clinical evidence strongly supports AKI as an MCC during this encounter. The payer auditor's downgrade applies the wrong standard — clinical resolution rather than coding classification. The creatinine trajectory, active interventions, medication adjustments, and physician documentation collectively establish that the condition was present, managed, and clinically significant. Combined with the CDI/coding team's guideline citations, this case has strong appealability based on the documented clinical record, subject to final coding validation and payer review.

Illustrative disputed value: \$12,000–\$18,000 depending on facility-specific DRG rate differential. Not a recovery guarantee. This is one case. The pattern — payer auditors applying clinical-resolution logic to downgrade MCCs — is systematic and likely affects other sepsis, respiratory failure, and AKI-complicated cases in the same audit batch.

OPERATIONAL NOTE

Prevention Note

For future similar cases: DRG downgrades in sepsis cases are frequently driven by documentation that describes the treatment provided without connecting clinical severity markers to the higher-weighted DRG. If the treating physician had documented the specific clinical indicators of organ dysfunction severity — hemodynamic instability, vasopressor requirements, mechanical ventilation parameters — in the discharge summary (not just the progress notes), the coding team would have had the documentation needed to support the higher DRG at initial billing, before payer audit.

Forward-looking operational insight for the client's physician and CDI teams.

INTERNAL PATTERN TAGS

Trend Tags

Internal trend tags shown for specimen purposes.

DRG DOWNGRADE

871 → 872

AKI AS MCC

AUDITOR: RESOLUTION ≠ CODING STANDARD

SEPSIS COMPLICATION

CLINICAL-CODING CROSSOVER

CDI COMPLEMENT

RECOVERY: STRONG

Want this level of reasoning on your own cases?

START A 3-CASE EVALUATION

SEND A DE-IDENTIFIED CASE

CASE SUITABILITY

Clinovian

This specimen demonstrates the DRG Downgrade Challenge Memo — the physician-level clinical severity argument that complements your CDI team's coding-rule citations. Paid-but-downgraded claims never appear in denial dashboards. They require someone looking for them.

[Start a 3-case evaluation →](#) · [Send a de-identified case →](#)

Clinical consulting only. Not a law firm. Coding decisions remain with the client's certified coding professionals.

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